

City General Hospital

Department of Internal Medicine — Endocrinology

Patient Name:	Rajesh Kumar	MRN:	MGH-2024-04471
Age / Sex:	54 years / Male	Date:	05-Mar-2026
DOB:	12-Mar-1970	Physician:	Dr. Ananya Sharma, MD

CLINICAL REPORT — OUTPATIENT VISIT

Chief Complaint: Increased thirst, frequent urination, fatigue, and blurred vision for the past 3 months.

History of Present Illness: Mr. Rajesh Kumar is a 54-year-old male with a 5-year history of hypertension who presents with polyuria, polydipsia, and unexplained weight loss of approximately 6 kg over 3 months. He reports waking up 3–4 times per night to urinate. He has a strong family history of diabetes mellitus (mother and maternal uncle). BMI is 31.2 kg/m².

VITAL SIGNS

Blood Pressure:	148/92 mmHg	Heart Rate:	82 bpm
Temperature:	37.1 °C	SpO2:	98%
BMI:	31.2 kg/m ²	Weight:	88 kg

LABORATORY RESULTS

Test	Result	Normal Range	Unit	Flag
Fasting Blood Glucose	214	70 – 100	mg/dL	HIGH ↑
HbA1c	8.7	< 5.7	%	HIGH ↑
Post-Prandial Glucose	318	< 140	mg/dL	HIGH ↑
Serum Insulin	22.4	2.6 – 24.9	μU/mL	Normal
Total Cholesterol	224	< 200	mg/dL	HIGH ↑
LDL Cholesterol	148	< 100	mg/dL	HIGH ↑
HDL Cholesterol	38	> 40	mg/dL	LOW ↓
Triglycerides	298	< 150	mg/dL	HIGH ↑
Serum Creatinine	1.1	0.7 – 1.2	mg/dL	Normal
eGFR	72	> 60	mL/min	Normal
Urine Microalbumin	82	< 30	mg/g	HIGH ↑
HbA1c (3 months prior)	7.2	< 5.7	%	HIGH ↑

CLINICAL NOTES

The patient's fasting glucose of 214 mg/dL and HbA1c of 8.7% are consistent with poorly controlled Type 2 Diabetes Mellitus. The progressive rise from 7.2% to 8.7% over 3 months indicates worsening glycaemic control. Elevated LDL and triglycerides indicate dyslipidaemia, a common comorbidity. Microalbuminuria (82 mg/g) suggests early diabetic nephropathy. Hypertension (148/92 mmHg) adds significant cardiovascular risk. Recommend initiation of Metformin 500 mg BD, dietary counselling, and ophthalmology referral for diabetic retinopathy screening.

PRELIMINARY DIAGNOSIS

Type 2 Diabetes Mellitus — Uncontrolled (HbA1c 8.7%). Associated: Dyslipidaemia, Early Diabetic Nephropathy, Hypertension.